

COPAY CARD IMPACT · CALCULATOR

Free Drug. Full Price Claim. Your Plan Pays.

A 5-step walkthrough to model how manufacturer copay cards redistribute cost from member to plan, plus an accumulator vs. maximizer decision matrix. The math is usually bigger than employers expect.

01 The Five-Step Calculation

The copay card transaction is real money moving in a direction most employers have not traced. Run these five steps in order, with your PBM data and your member cost-sharing structure open. Step 3 is where the number usually lands harder than expected.

STEP 1 · DATA

Identify copay card utilization in your data.

Request from your PBM: drugs with active manufacturer copay card programs in your formulary, members using copay cards on each drug, and total claims dollars where copay assistance was applied. If your PBM cannot identify copay card utilization, document that answer; the inability to report on this is itself diagnostic.

STEP 2 · CURRENT STATE

Determine your accumulator status.

For each drug with copay card utilization, find out whether your plan uses an accumulator, a maximizer, or nothing. With no program, copay card amounts count toward the member's accumulators by default; the member hits OOP max faster, the plan pays everything sooner. That is your baseline.

STEP 3 · PER-DRUG MATH

Model the per-drug financial impact.

For each high-cost drug with copay card utilization, gather: annual drug cost per member, member copay or coinsurance per fill, copay card value per fill (often public from the manufacturer), fills per year, and the member's deductible and OOP max. Calculate how quickly the card pushes the member past OOP max. After that point, the plan pays 100%.

STEP 4 · PLAN-LEVEL AGGREGATE

Estimate plan-level impact.

Aggregate across every drug with a copay card program. Calculate the total additional plan cost from accelerated OOP max accumulation versus the scenario where members paid their cost-sharing out of pocket without a copay card. The difference is the cost redistribution from member to plan, in dollars.

STEP 5 · DECISION

Evaluate your options.

No accumulator or maximizer? Evaluate implementing one. Using an accumulator? Assess member impact when cards exhaust mid-year. Using a maximizer? Confirm your PBM is coordinating correctly. For any approach, identify the five to ten drugs with the largest copay card financial impact and start there.

Illustrative example for educational purposes. Actual amounts vary by plan based on drug mix, member population, and copay card programs in effect.

03 Accumulator vs. Maximizer · Decision Matrix

Once Step 5 surfaces the financial impact, the design choice falls into three positions plus an increasingly common hybrid. Each carries different implications for plan cost, member experience, and PBM administration. The right answer depends on drug mix and member population.

POSITION A · NO PROGRAM

Most expensive scenario for the plan.

Copay card dollars count toward deductible and OOP max by default. Members hit OOP max faster than without the card; plan absorbs full cost from that point through year-end. The baseline: if you have not actively chosen accumulator or maximizer, you are here.

POSITION B · COPAY ACCUMULATOR

Blocks the manufacturer's payment from counting.

Manufacturer keeps paying the copay; member accumulators do not advance. When the card runs out, members face full cost-sharing. Some switch to generics; others stop therapy. Saves the plan money; disrupts members on specialty therapies. Best fit when alternatives exist in-class.

POSITION C · COPAY MAXIMIZER

Restructures cost-sharing to capture full card value.

Member cost-sharing is set at the maximum the card will cover; card pays it; member pays nothing out of pocket. Plan avoids the cost it would otherwise absorb under Position A. Preserves member experience but requires sophisticated PBM administration. Not every PBM coordinates this correctly.

HYBRID · INCREASINGLY COMMON

Maximizer on most categories, accumulator with carve-outs.

Maximizers on most copay-card-eligible drugs; accumulators on categories with clear alternatives; carve-outs for high-risk therapeutic areas (oncology, transplant, severe autoimmune). Requires a capable PBM and clinical criteria for the carve-out list.

04 Implementation Watch-Outs

Math points to a position. Implementation determines whether the plan captures projected savings without disrupting members. Four failure modes to plan around:

§ **Member communication 60 days before go-live**

Members on copay-card-supported therapies need to know what is changing and their OOP exposure when the card exhausts. One letter is never enough; build multi-touch communication with prescriber outreach for high-volume drugs.

§ **PBM execution capability check**

Maximizer programs depend on real-time coordination between cost-sharing structures and manufacturer card values. Confirm prior implementations, request client references, require performance guarantees in writing. Not every PBM does this well.

§ **Carve-out list aligned with clinical criteria**

For a hybrid, build the carve-out list from clinical criteria, not from a PBM-supplied marketing list. Engage your clinical advisory team in the definitions.